



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : محروسه سيد ابراهيم حسين

العنوان : الوراق

الحي / المدينة : الوراق

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 29106302101266 رقم التليفون : 01060347451

العمر (تاريخ الميلاد) : 30-06-1991 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة

قسم العلاج :
الأورام

[11129]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: محروسه سيد ابراهيم حسين National ID: 29106302101266	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

١٣٢٠





٢٠٢٠/١٢ ٢٨١٠٢٣٠٢١٠٢٢١١
دولة فلسطين

مخرجة

مسجلة

حصة الميراث وحصة الميراث

الطبعة سارية المفعول ٢٠٢٧/١٢/٢٨



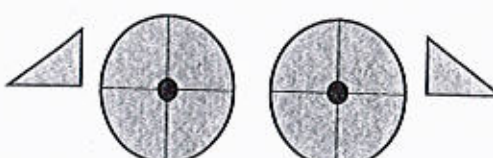
Name: محروسه سيد ابراهيم			
MR. : 937			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
	بشأن تقرير طبي للمريض		إيماء للطلب المقدم من
	رقم طبي:		بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
	تاريخ وساعة الخروج :		تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه MRI BRAIN FINDINGS SUGGESTIVE TO OLFACTORY GROOVE MENINGIOMA FOR NEUROSURGERY CONSULTATION LT MRM AT 23/11/2025 YPT0N0 C7 PHESGO 600 SC PORT REPORT GOR PORT DONE MDT 14/1/2026 PORT ADG PHESGO ADJUVANT TAMOXIFEN 20 TAMOXIFEN 20 MG CALCIUM ONE ALPHA 9/2/2026 MRI + C: Large lobulated frontal extraaxial lesion, + moderate edema (meningioma) For neurosurgical opinion ADJ PHESGO C8 ADJUVANT TAMOXIFEN 20 TAMOXIFEN 20 MG CALCIUM ONE ALPHA echo next cycle phesgo 9 and 10 and 11 and 12 cycle + TAM clinical pharmacy note : for c9 phesgo + tam new echo 18/2/2026 68% for c11 phesgo + tamphen pt developed menangioma and underwent excision for mdt PT NOT RECEIVED PORT YET Received PORT at Cairo University 40 Gy 15 Fraction from 19/7/2026 till 8/8/2026. Planned for RTH for brain meningioma as has residual (as per MDT decision) MRI Brain 30/7>. Operative bed mid subfrontal extra axial dural based small enhancing lesion likely local lesion residue. Due for C12 phesgo and for 3 more months Tam , new echo requested Pharmacy Visit For C11 Phesgo Patient recived last Phesgo in April The patient is currently scheduled to receive C12 of PHESGO and will continue tamoxifen for an additional three months, with a new echocardiogram requested.			
تحريراً في ,			
	الوقت :		التاريخ : 21-09-2026
		ايه مدحت بدوي محمد	الطبيب المعالج :
يعتمد ,			

MDT FORM

1411 / 2026 G

Name	مروة سيد ابراهيم	MRN	95012025
Age	34 y	ID	٢٩١٠٦٢٠٩١٠١٩٧٧
Menopausal status	pre	Phone	01152367773
NRS SCORE		Marital status	married
BMI		Family history	✓
CO morbidity	NAD	REASON OF MDT	Post op

Clinical Data

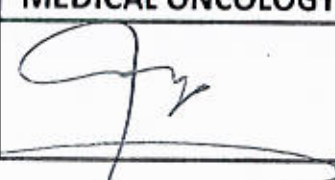
BX:	IDC			R	L
ER:	PR:	HER 2:	Ki67:		
218	018	3	38-10		
Sonomammography:					
Mets Workup:				progressive TNM T4b N2 M	
(NB) Ptn complains of loss of vision post op. MRI brain → Requested.					
Recommendations:				Post NACT Lt MRN No residual tumor Free margin LN (0112) To No صبر عبالق ١٢/١٢	

MDT OUTCOME & PLAN

for: PORT
adj. Fresgo ly
± adj. Tamex 5y after discussion of pt.
low benefit

SIGNATURE

f. U MRI brain.

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Patient Name : الأستاذ / محروسه سيد ابراهيم حسين	Request No. : 13001222
Patient No. : 950-2025	Request Date : 04/11/2025
Path No. : 3379/25	Verification Date : 23/11/2025
Sex : Female	Financial Category : جوسفان رومني - نفقة الدولة
Age : 34 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 26x26x7 cm, with covering skin ellipse measured 23x17 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 6x5x3 cm, located 3 cm from the least deep margin.
- * Axillary fat measured 7x6x2 cm, entangling 12 lymph nodes, the largest was 1.5x1x0.5 cm.
- * 13 Blocks:
- 1:6- T.B.S., 7- N&A., 8- Deep margin., 9:13- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy with microcalcifications.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Twelve lymph nodes free of viable metastatic deposits, LNs (0/12) (Multiple lymph nodes showing effect of therapy).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

- Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed tumor.
 - Ductal carcinoma in situ: Absent.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذة / محروسة سيد ابراهيم حسين	Request No. : 13001222
Patient No. : 950-2025	Request Date : 04/11/2025
Path No. : 3379/25	Verification Date : 23/11/2025
Sex : Female	Financial Category : جرسات روسي - نفقة الدولة
Age : 34 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 26x26x7 cm, with covering skin ellipse measured 23x17 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 6x5x3 cm, located 3 cm from the least deep margin.
- * Axillary fat measured 7x6x2 cm, entangling 12 lymph nodes, the largest was 1.5x1x0.5 cm.
- * 13 Blocks:
1:6- T.B.S., 7- N&A., 8- Deep margin., 9:13- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy with microcalcifications.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Twelve lymph nodes free of viable metastatic deposits, LNs (0/12) (Multiple lymph nodes showing effect of therapy).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

- Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed tumor.
 - Ductal carcinoma in situ: Absent.

Verified by :

Dr.Duaa Mohamed

Patient Name	: الأستاذ / محروسه سيد ابراهيم حسين	Request No.	: 13001222
Patient No.	: 950-2025	Request Date	: 04/11/2025
Path No.	: 3379/25	Verification Date	: 23/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 34 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Nipple & areola: Free.
- Deep Margin: Free.
- Axillary lymph nodes: Twelve lymph nodes free of viable metastatic deposits, LNs (0/12) (Multiple lymph nodes showing effect of therapy).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed



Presidential Initiative for Women's Health - Pathology Lab
مبادرة الرئاسة الصحية للمرأة - مختبر الأمراض

Patient Name : الأستد / مبروكه عبد الهادي حسن	Request No. : 13005157
Patient No. : 29106302101266	Request Date : 08/03/2025
Path No. : 950/25	Verification Date : 19/03/2025
Sex : Female	Financial Category : برنامج الرعاية الصحية - مبروكه
Age : 33 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 6/3/2025.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis) (3+3+2) Grade III.
- Ductal carcinoma in situ: No.
- Perineural invasion: No.
- Angiolymphatic invasion: No.
- IPT (Positive control showed appropriate reactivity):
- ER: Faint nuclear reaction in 1% of tumor cells (score 2/8).
- PR: No nuclear reaction in the tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 38% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy.
Invasive duct carcinoma, Grade III.
IPT:

- ER: Negative (score 2/8).
- PR: Negative (score 0/8).

Verified by:
DR. Menna Nabil

برقية
مبروكه عبد الهادي حسن
6/3/2025

Printing Date: 25/03/2025 16:42:35

Page 1 of 1

Printing By: Mrs. Mayada Hegazy

القاهرة (8) - الدور الثاني - مبنى رقم (257) - أول طابق الجناح - مبنى القابلية - الطابق 1 - مصر
Rosa No. (8) - First Floor Building No. 257 - Nile Corridor - Main Fl. Qadima - Cairo, Egypt

Patient Name : الأستاذة / معروفة مينة محمد هادي
Patient No. : 29106302101266
Path No. : 950/25
Sex : Female
Age : 33 Y
Referring Doc :

Request No. : 13009157
Request Date : 08/03/2025
Verification Date : 19/03/2025
Financial Category : زوار - مريض
Department : Out Patient
Registered by : Mrs Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).
- Ki67: Positive in 38% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations. proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0 none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar,
Dr. Nermeen Mohammed.

Verified by:
DR. Menna Nabil

Printing Date : 25/03/2025 10:42:35

Page 2 of 2

Printing By : Mrs Mayada Hegazy





Patient Name: Mahrousa Sayed Ibrahim Hussein	Date : 27/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 33 Year(s)
Medical ID: 29106302101266	Pat. ID: 98005

Combined bilateral breast mammography and complementary ultrasound

Indications:

left breast palpable lumps.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Reports:

- Scattered fibroglandular density (ACR B)
- Left breast shows lower central and lower inner multiple irregular mass lesions, of high density.
- Complementary ultrasound revealed:
- Left breast shows lower central and lower inner multiple irregular spiculated hypoechoic mass lesions seen as the following:
 - Left 6 o'clock measured 29x13 mm seen at 5mm from the nipple.
 - Left UQ at 9-10 o'clock measured 23x17mm with impending skin invasion seen at 2.5 cm from nipple
 - Another few scattered satellite lesions are noted extended from 5 to 11 o'clock largest measured 15x14mm
- Few left axillary pathological level I LNs showing muffled hila, measuring 1.1x1.6cm, with cortical thickness reaching 6mm.
 - Free left supra and inframammary regions
- Right breast: no suspicious solid mass lesions
- Normal skin thickness, and nipple-areola complex of the right breast
- Nonspecific right axillary LN.

Opinion:

- Left breast multifocal malignant looking mass lesions extending from 5 to 11 o'clock [BIRADS 5] for histopathological correlation.
- Pathological left axillary LNs level I.
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. basma mohamed

27/02/2025 3:58 PM



Patient Name: Mahrousa Sayed Ibrahim Hussein	Date : 08/04/2025
Exam : C.T ABDOMEN & PELVIS	Age : 33 Year(s)
Medical ID: 950/2025	Pat. ID: 98005

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I. Chest:

- Left breast UIQ irregular masses are noted the largest is measuring 3.9 X 4.6 cm.
- Enlarged left axillary lymph nodes at level I and II reaching 1.5 X 3.5 cm.
- Prominent left internal mammary lymph nodes reaching 8 X 10 mm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- Middle lung lobe thick atelectatic bands.
- Prominent subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II. Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- IUD is seen.
- No sizable worrisome osseous lesions.

Opinion:

- Reported case of left breast cancer revealed:
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Dr. Mohamed Momen, M.D.

Patient Name: Mahrousa Sayed Ibrahim Hussein	Date : 12/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 34 Year(s)
Medical ID: 950/2025	Pat. ID: 98005

Bilateral high resolution breast sonography revealed:

Clinical summary: 34-year-old female left breast multicentric malignant process underwent NACT, follow up study.

Findings:

Right breast:

- Preserved fibroglandular pattern.
- No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast:

- Preserved fibroglandular pattern.
- Multiple irregular anti-parallel speculated heterogenous solid mass lesions with moderate internal vascularity are seen at :
 - Index mass; 6 o'clock, 10 mm from nipple, 9 mm deep to the skin measuring 20 x 17 mm (regressive course).
 - Multiple masses are seen 9 o'clock, all are more than 1 cm deep to the skin and are more than 1.5 cm from nipple, largest among them is posterior third measuring 18 x 18 mm.
 - 3-11 o'clock, Multiple variable sized satellites also more than more than 1.5 cm from nipple and more than 1 cm deep to the skin largest seen at 8 o'clock measuring 10 x 11 mm.
- Normal skin/nipple/areola complex.
- Three remaining level 1 pathological axillary LNs , largest measured 20 x 17 mm with lost hilum. No infra or supraclavicular lymph nodes.

Patient Name: Mahrousa Sayed Ibrahim Hussein	Date : 12/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 34 Year(s)
Medical ID: 950/2025	Pat. ID: 98005

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): Multicentric malignant process with ipsilateral pathological axillary lymph nodes (level 1) showing partial radiological response.

Best Regards,

Dr. Amal Hegazy, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
AM 11:48 2025/08/12

Imaging report



الاسكان
CAIRO SCAN
1974

Name : Mrs. Mahrosah Sayed Ibrahim Husseln

Date : 05/04/2025

Code : 3283789

Tc-99m MDP DUAL PHASE BONE SCAN:

PROCEDURE:

- Early blood pool and delayed osseous phase whole body scans were obtained 5 minutes and 3 hours respectively post tracer injection.

FINDINGS:-

- No abnormal focal active or cold osseous lesions could be detected all over the scanned skeleton to account for serious osseous lesions including metastases.
- Degenerative changes at multiple spine levels are noted.
- Exaggerated osteoarthritic changes around both shoulders and knees are also observed.

OPINION:

The skeleton is free from any scintigraphically detected serious osseous lesions including metastases.

Best regards
Dr. Barna El-Kholy, MSc.
Asst. Professor of Nuclear Medicine (NCI)
Cairo University

For any inquiries regarding the findings in the report please contact us on:
radreport@cairoscan.com.eg

ECHO REPORT

NAME: **درويه سيد ابراهيم** DATE: 28/10/2025
ID NUMBER: 90/50 SEX: F AGE: 35

EF	65%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.3	3.5-5.8cm	AO.R	2.6	3-3.7cm
LVESD		2-4cm	LA	3.4	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

داليا عمار